Housing First or Treatment First?

Ask the Kids, Not the Parents

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Job market presentation

The opinions expressed are the author's and not necessarily those of CMHC.

Motivation: should housing assistance require sobriety?

- Homelessness in the United States is at a record: 771,480 people in shelters or on the street in the January 2024 point-in-time count
- July 2025: Executive Order 14321 directs federal agencies to end support for Housing First, which "fails to promote treatment, recovery, and self-sufficiency"
- HUD's notice would have capped permanent housing at 30 % of renewal funding, down from over 80 %, until a federal court struck it down (June 2026)
- The question is live and has two answers in the policy debate

Sources: HUD PiT 2024; Executive Order 14321; HUD FR-6900-N-25; State of Washington v. HUD (2026).

Two ways to give housing assistance

Housing First

- Eligible low-income households that draw a voucher keep it
- No requirement to stop using drugs
- The benchmark: most US permanent housing funding works this way

Treatment First

- Assistance conditional on not using drugs and, for those with a substance use disorder (SUD), on being in treatment
- Enforced by an imperfect drug test: some users go undetected

The debate: two claims about the recipients

- **Critics of Housing First**: unconditional assistance creates moral hazard, weaker incentives to stay sober and employed
  - More drug use and SUD → more homelessness → a cost borne by those who fund the programme
- **Critics of Treatment First**: conditionality delays stable housing for those who need it most; recovering while homeless is harder
  - The circle runs the other way, so Housing First even improves the budget

Both claims are about the households that receive the assistance

Neither is about their children

Eide (2020); National Alliance to End Homelessness; Tsemberis et al. (2004); Larimer et al. (2009).

What randomised trials show, and what they cannot

They find

- Large, persistent gains in housing stability
- Mixed-to-null effects on drug use
- Lower mortality of homeless veterans with mental illness (Costantini, 2025)

They cannot see

- Beyond 6 to 36 months
- The children's adult lives
- Households outside the trial
- The government's budget

Only one small trial compares a sobriety condition with unconditional housing (Milby et al., 2005)

$\Rightarrow$ a structural model can do all four

Tsemberis et al. (2004); Padgett et al. (2006); Goering et al. (2014); Aubry et al. (2016); Gubits et al. (2016, 2018).

This paper

A quantitative life-cycle model in which drug use harms the user's child

- Forward-looking households decide whether to use drugs, knowing the risk of a SUD
- Children share their parent's consumption and dwelling and inherit an earnings penalty; parents value them imperfectly ($\omega_c < 1$)
- Homelessness, rationed means-tested vouchers, serious mental illness (SMI), treatment
- General equilibrium: the drug price clears its market; the voucher probability π_V balances the budget
- Transition from Housing First to Treatment First over every cohort alive at the reform

One reform, taken apart: moral hazard, three externalities on children, and general-equilibrium effects, each switched on separately

Main findings

1. **Moral hazard alone does not justify conditionality.** Without effects on children, a newborn loses 1.09 % of lifetime consumption under Treatment First, although drug use falls by 28 %
2. **The externalities on children do.** With them, and with the drug price and π_V clearing, Treatment First cuts drug use by 21 % and homelessness by 11 %, and a newborn gains 2.57 %; three quarters of it is an externality the parent does not count. Parents gain too, 0.69 %
3. **Childhood homelessness falls**, by 11 %, although critics expect conditionality to raise it
4. **Transition**: homelessness never rises; children born at the reform gain 1.45 %. If those with a SUD cannot stop using, the early rise depends on design

Mechanism: three externalities on the child

A parent counts only the share ω_c of what its choices cost its child

1. **Budget externality.** Spending on drugs, and earnings lost while a SUD keeps the parent unemployed, are consumption and housing the child goes without
2. **Earnings externality.** A child whose parent had a SUD while it lived at home starts adult life with lower productivity
3. **Childhood homelessness externality.** Denying assistance to users exposes more of them to the shelter, and the child lives where its parent lives

Housing First leaves the first two in place; critics of Treatment First worry about the third

$\Rightarrow$ in the model, Treatment First reduces all three

Who uses drugs matters, and the literature needs preferences to get it

- 8.90 % of adults aged 26 and over have a drug use disorder of any severity (NSDUH 2024): most do not use
- The average user's income is 0.74 of the population average: use sits at the bottom
- Existing models reproduce both facts by letting preferences for drugs differ by education (Greenwood et al., 2025)
- Here: drugs are a luxury good, and use concentrates at the bottom because the mortality risk of a SUD is uninsurable; no preference varies with income

Why it matters: vouchers are means-tested, so whose children bear the costs depends on who uses

Related literature

- **Housing First trials**: Tsemberis et al. (2004), Goering et al. (2014), Aubry et al. (2016), Costantini (2025)
  → reproduce their micro effects, then add the macro effects they cannot observe
- **Deterrence of drug testing**: Mehay and Pacula (1999)
  → disciplines how strongly the screen deters use
- **Economic models of drug use**: Becker and Murphy (1988), Greenwood et al. (2025)
  → an income gradient from uninsurable mortality risk, not preferences
- **Homelessness in equilibrium**: İmrohoroğlu and Zhao (2022), Abramson (2026)
  → add endogenous drug use, disorder and recovery
- **Intergenerational transmission and parental altruism**: Nilsson (2017), Altonji et al. (1997)
  → price the externalities on children inside the model

Roadmap

1. Introduction
2. A stylized model: who uses drugs
3. The quantitative model
4. Calibration and validation
5. Long-run results
6. The transition
7. Robustness and conclusion

Stylized model: environment

- Two periods of thirty years (ages 26 to 85); productivity y lognormal, mean one, fixed for life
- Everyone works in period 1; in period 2 out of work with probability u_0, or $u_1 > u_0$ with a SUD
- Save at zero interest in period 1, no borrowing; food assistance $F(ye) = \max\{0, \bar{c} - \varrho\, ye\}$

$$X(c,v) = c^{1-\theta}(b+v)^{\theta}, \qquad u(c,v) = \frac{X^{1-\sigma}}{1-\sigma} + \aleph$$
$$\pi(v) = \pi_d \mathbf{1}\{v > 0\}, \qquad \phi(0) = \bar{\phi}, \quad \phi(1) = \bar{\phi}^{\kappa_{\phi d}},\ \kappa_{\phi d} > 1$$

- $b > 0$: the first unit's marginal utility is finite, so most households use none
- A SUD brings two risks: dying (mortality channel) and losing one's job (unemployment channel)

Stylized model: the decision to use

Second period, with savings a and SUD status d:

$$W(a, y, d) = (1 - u_d)\, U\big(X(y + a + F(y), 0)\big) + u_d\, U\big(X(a + \bar{c}, 0)\big) + \aleph$$

First period, not using and using:

$$V^0(y) = \max_{a \geq 0} \Big\{ U\big(X(y - a + F(y), 0)\big) + \aleph + \beta \bar{\phi}\, W(a, y, 0) \Big\}$$

$$V^1(y) = \max_{a \geq 0,\, v > 0} \Big\{ U\big(X(y - a + F(y) - p_v v,\, v)\big) + \aleph + \beta \Big[(1 - \pi_d)\, \bar{\phi}\, W(a, y, 0) + \pi_d\, \bar{\phi}^{\kappa_{\phi d}}\, W(a, y, 1) \Big] \Big\}$$

A household uses if $V^1(y) > V^0(y)$

Stylized model: calibration

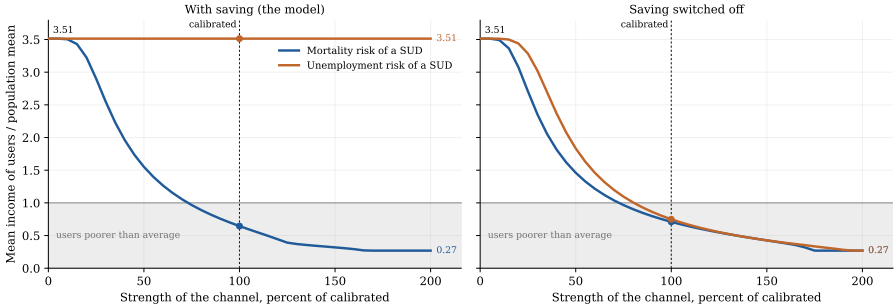
Set externally

Parameter	Description	Source or rationale	Value
β	Discount factor over a thirty-year period	0.995 annually, compounded	0.86
σ	Risk aversion	Standard; Attanasio and Weber (2010)	2
$\bar{\phi}$	Survival over a period, no SUD	US life table 2010, Arias (2014), ages 26–55	0.93
π_d	Chance that use starts a SUD	NSDUH 2024: SUD rate over participation	0.41
σ_y	Dispersion of log productivity	SCF 2022: bottom half holds 17.2% of earnings	0.95
u_0	Unemployment rate in period 2, no SUD	BLS CPS, average 2014–2024	0.049
u_1	Unemployment rate in period 2 with a SUD	NSDUH 2024, ages 26 and over	0.16
ϱ	Food assistance taper	SNAP benefit reduction rate	0.30
p_v	Price of a unit of drugs	Normalisation	1.00

Calibrated internally: five parameters, matched exactly; $\kappa_{\phi d}$ is pinned by users' income

Parameter	Target moment	Data	Model	Value
$\aleph$	VSL over income	80	80.0	8.64
b	Drug participation, NSDUH 2024	8.9%	8.9%	0.032
θ	Drug spending over users' income	16.0%	16.0%	0.22
$\kappa_{\phi d}$	Users' income over the average, NSDUH 2024	0.74	0.74	3.56
$\bar{c}$	Food assistance over labour income	0.42%	0.42%	0.10

Result: only the uninsurable risk reverses the income gradient



- No consequences: the average user earns **3.51** times the mean (a luxury good)
- Mortality alone: **0.64** at calibrated strength (crosses the mean at 73 %); unemployment alone: stays at **3.51**
- Without saving, unemployment alone also reverses it: **0.75**

Why: a penalty rises with income only if it cannot be insured

- The utility cost of the drug falls with income: a richer household gives up less marginal utility per dollar
- Losing a job: a richer household saves more to protect itself, so its loss is roughly proportional to its income $\Rightarrow$ the same utility cost for rich and poor
- Dying: wipes out the whole continuation value of being alive, which rises with income $\Rightarrow$ deters the rich, not the poor
- Life insurance protects descendants, not the household's own life
- Without saving, job loss also grows with income, which is why it then reverses the gradient as well

Quantitative model: environment

Keeps the stylized model's preferences and the two risks of a SUD, and adds:

- Annual periods, life cycle, income and unemployment risk, unemployment insurance, retirement, progressive taxes
- Housing: a free shelter or street h_1 and three rental sizes $h_2 < h_3 < h_4$; rationed, means-tested vouchers
- A drug-use decision every year; recovery; costly, publicly funded treatment τ
- SMI, likelier with a SUD; it lowers job finding and recovery and raises mortality
- Homelessness raises mortality, worsens employment and lowers recovery
- Children who live with their parent for J_K years
- An endogenous drug price; a budget held at its benchmark net revenue

State (j, a, y, e, d, m, η), with $d \in \{\text{never, recovered, current}\}$ and voucher standing $\eta \in \{\text{none, holder, excluded}\}$

Preferences: the child shares the bundle but not the drug

$$X_P = \left[\left(\frac{c}{\xi(j)}\right)^{1-\gamma}\left(\frac{h}{\xi(j)}\right)^{\gamma}\right]^{1-\theta}(b+v)^{\theta}, \qquad X_c = \left[\left(\frac{c}{\xi(j)}\right)^{1-\gamma}\left(\frac{h}{\xi(j)}\right)^{\gamma}\right]^{1-\theta} b^{\theta}$$

$$u(c,h,v,\tau) = U(X_P) + \omega_c\left(\xi(j)-1\right)U(X_c) + \aleph - \tau\,\bar{\tau}^{u}$$

- Equivalence scale $\xi(j)$: two members while the child is at home, one after
- $\omega_c < 1$: the parent counts only part of its child's welfare
- No preference parameter varies with the SUD, SMI or housing: the disorder works only through what it costs

The SUD, recovery and the child's penalty

$$\Pr(d' = 2 \mid d \neq 2, v) = \pi_d \, \mathbf{1}\{v > 0\}$$

$$\pi_r(h, m, \tau) = \min\left\{1,\ \bar{\pi}_r \, \mu_H^{\mathbf{1}\{h=h_1\}} \, \mu_m^{\mathbf{1}\{m=1\}} \, g^{\mathbf{1}\{\tau=1\}}\right\}$$

Child's productivity at entry:

$$\log y_1^{\text{child}} = \rho \log y^{\text{parent}} + (1-\rho)\mu + \epsilon + \log(1-\delta_d)\,\mathbf{1}\{\text{parent had a SUD while the child was at home}\}$$

What the parent's disorder costs the child, in its own lifetime value:

$$D(y, d) = \mathbb{E}\left[V(1, s_{\text{child}}) \mid y, 0\right] - \mathbb{E}\left[V(1, s_{\text{child}}) \mid y, d\right] \geq 0$$

The household's problem

$$
\begin{aligned}
V(j,s) = \max_{\{h,v,\tau,a'\}} \Big\{ & U(X_P) + \omega_c(\xi(j)-1)\,U(X_c) + \aleph - \tau\bar{\tau}^u \\
& + \beta\,\phi(j,h,d,m) \int V(j{+}1,s')\,\Gamma(\mathrm{d}s' \mid s,h,v,\tau,a') \\
& - \beta\,\chi(j)\,\omega_c\,D(y,d) \Big\} \\
\text{s.t.} \quad & c + p_v v + a' = y^n + a + T_0 \mathbf{1}\{j=1\} - p_r h\,\mathbf{1}\{h \neq h_1\} + Vch + F
\end{aligned}
$$

- $\chi(j)$: the chance the child leaves home after this period, at the parent's death before J_K or at J_K
- The parent is charged $\omega_c D(y,d)$ when its child leaves; the rest of D is an externality

Vouchers and the two policies

- Eligible while $\hat{y} < \kappa_V \, y^{\text{med}}$; an eligible household without a voucher draws one with probability π_V; a holder keeps it while eligible
- **Housing First**: that is the whole rule
- **Treatment First**: holders and new draws are tested; a household fails if it uses, or has a SUD and is not in treatment
  - Treatment is publicly funded; no one may use, even in treatment (Executive Order 14321 and its guidance)
  - A failure is caught with probability $p_{\text{det}} < 1$; each test costs c_{test}
  - A caught household loses the voucher and is excluded, returning to the lottery with probability ψ_x a year
- p_{det}, ψ_x and c_{test} bind only in the experiment

Government, drug market and equilibrium

Drug market: the price clears a supply curve

$$p_v = \nu_1 S_v^{1/\varepsilon_s}, \qquad S_v = \sum_j \int v(j,s)\, \mathrm{d}\Phi(j,s)$$

Government: labour and capital income taxes and estates of the dead fund UI, social security, food assistance, vouchers, drug tests and treatment; tax rates are held fixed

Equilibrium: at every date, households optimise, the distribution follows its law of motion,

- p_v clears the drug market
- π_V holds net government revenue at its benchmark level

Either can be switched off: this separates the micro effects a trial sees from the macro effects it cannot

Calibration strategy

- 59 parameters: 38 set externally from data or the literature, 21 calibrated internally
- 26 carry the story, in six blocks: who uses drugs; costs of homelessness and vouchers; SMI; recovery and treatment; externalities; the drug market
- The other 33 are standard life-cycle parameters
- Three parameters bind only in the experiment: detection p_{det}, exclusion ψ_x, test cost c_{test}
- Every target and its dataset, year and universe are documented in the replication package

Selected parameters and targets

Parameter	Moment and source	Data	Model	Value
b, first unit's marginal utility	Any drug use disorder, NSDUH 2024	8.90%	8.96%	0.0054
π_d, chance use leads to a SUD	Share of users with a SUD, NSDUH 2024	41.3%	42.6%	0.0345
$\kappa_{\phi d}$, mortality with a SUD	Users' income over average, NSDUH 2024	74%	62%	4.00
$\aleph$, value of being alive	VSL, Viscusi and Aldy (2003)	80	81	87.6
h_1, service flow of homelessness	Homelessness, HUD PiT and Burt et al. (2001)	0.40–1.00%	0.62%	0.0216
π_V, chance of drawing a voucher	Recipients over eligible, HUD 2023	26.6%	27.5%	0.0675
μ_H, recovery when homeless	Abstinence, housed/homeless, Milby et al. (2005)	1.47	1.45	0.696
$\bar{\tau}^u$, utility cost of treatment	Treatment take-up, NSDUH 2024	24.5%	24.9%	4.49
ω_c, parent's weight on its child	Two-adult over lone-parent use, NSDUH 2024	17.5%	17.7%	0.085
δ_d, child's earnings loss	Earnings lost in utero, Nilsson (2017)	0.24	n.a.	0.24
ε_s, supply elasticity	Mace et al. (2020)	1.46	n.a.	1.46

Table 3 of the paper; Model is the Housing First benchmark.

Disciplining the externalities

Parent's weight on its child, $\omega_c = 0.085$

- NSDUH 2024: an unmarried adult living with another adult and no child is 17.5 % more likely to use than a lone parent with one child, reweighted to the same age, sex, marital status and income
- The model matches it as the rise in use when the child gets no weight (17.7 %)
- At Nishiyama's (2000) 0.69, Treatment First still raises a newborn's welfare, by 3.18 %

Child's earnings penalty, $\delta_d = 0.24$

- The earnings loss at age 32 that Nilsson (2017) measures for children of mothers under 21 exposed in utero to a Swedish alcohol experiment
- The earnings externality is only 18 % of the gain; with no penalty Treatment First still gains

Disciplining the screen

Detection rate p_{det}: set so the screen deters as much as the closest evidence

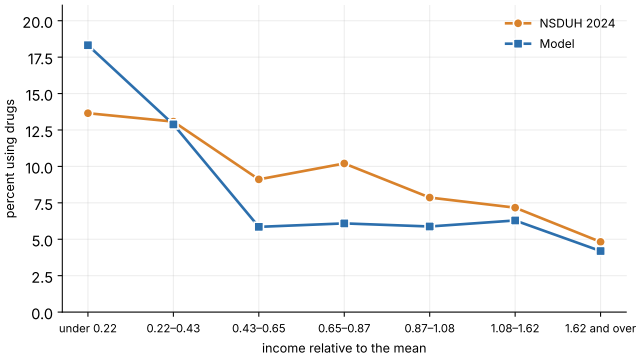
- US military random drug testing with dismissal: those who stayed were 61 % less likely to use marijuana (Mehay and Pacula, 1999)
- In the model, with $p_{\text{det}} = 20\,\%$, 62 % of the users facing the screen stop when it is introduced, and none starts
- Compared as proportional responses, since the military measure is broader
- At half the rate, 41 % stop, inside the 11 to 45 % range once leavers are counted; a newborn still gains 1.65 %
- A test costs $35 (Texas Legislative Budget Board, 2013), the upper end of the estimates; exclusion lasts three years on average

Validation: untargeted moments

Moment	Data	Model	Source
Abstinence, contingent over unconditional housing, ratio	1.26	1.40	Milby et al. (2005)
Three-year mortality, households with SMI put on the street, pp	4.6	3.6	Costantini (2025)
Child of a SUD parent acquires a SUD, odds ratio	1.44, 2.51	1.46	Mellentin et al.; Yoon et al.
Children living with a parent with a SUD, %	4.6	5.8	NSDUH 2024
Unemployment given a SUD, %	16.2	14.5	NSDUH 2024
Drug use with SMI over without, ratio	2.44	2.64	NSDUH 2024
Participation price elasticity	-0.3 to -0.9	-1.32	Saffer and Chaloupka (1999)
Intensive price elasticity, users	-0.3 to -0.8	-0.63	Grossman and Chaloupka (1998)
Homeless adult had a homeless parent, odds ratio	1.8 to 2.4	6.23	Bassuk et al. (1997)
Employed share of the homeless, %	46	21.4	Meyer et al. (2024)

Table 4 of the paper.

Validation: drug use by income



- Only users' average income is targeted
- Bottom band: 18.3 % use in the model, 13.7 % in NSDUH 2024
- Top band: 4.2 % against 4.8 %
- Middle: flatter and lower, 5.9 to 6.3 % against 7.2 to 10.2 %
- Vouchers are means-tested, so the lower users sit, the more of them, and of their children, the screen reaches

NSDUH 2024: adults 26+ with a SUD of any severity, by personal income band relative to the mean of band midpoints.

Experiments

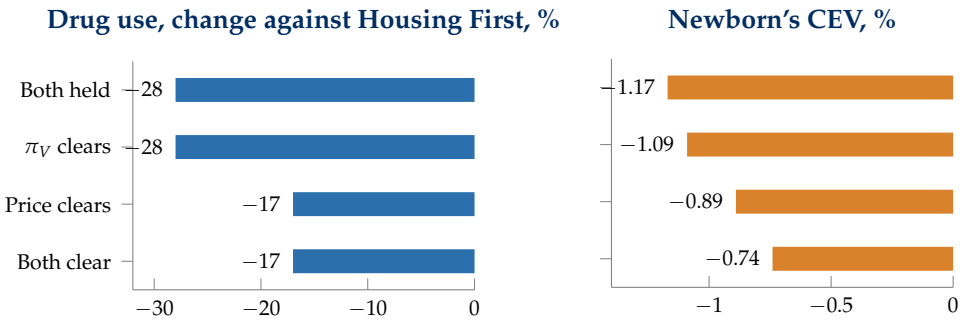
Replace Housing First with Treatment First; keep the programme rationed and taxes fixed

1. **Moral hazard alone**: children removed from welfare and from the parent's preferences, their productivity independent of the parent
2. **With the children**: the calibrated model, in three steps: the screen alone; then π_V clears the budget; then the drug price clears too (the main experiment)
3. **Transition**: every cohort alive at the reform, with p_v and π_V clearing every year

Welfare: consumption-equivalent variation (CEV) of a newborn, childhood included

$$W_t = \underbrace{\textstyle\sum_{j=1}^{J_K} \beta^{j-1}(\xi(j)-1)\,\mathbb{E}\,U(X_c)}_{\text{childhood}} + \underbrace{\beta^{J_K}\,\mathbb{E}\,V(1,s)}_{\text{adult life}}$$

Experiment 1: moral hazard alone does not justify conditionality



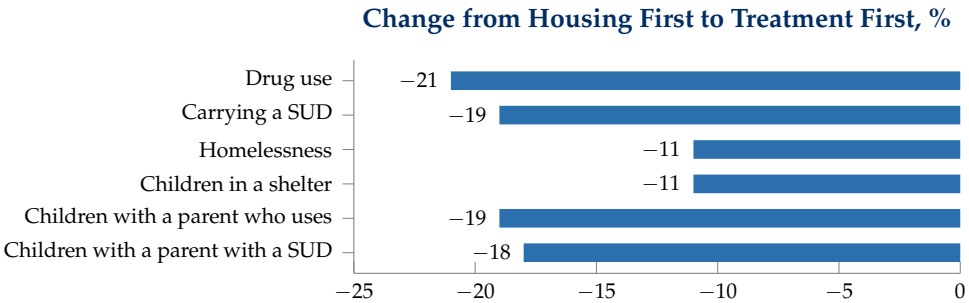
- Rows: the drug price and π_V held at their Housing First levels or allowed to clear
- Use falls by up to 28 %, yet a newborn loses in every case: the screen removes an option from households that weigh the risks correctly, and the budget gives little back (with π_V clearing, 58 % gain, by 0.05 % on average; 96 % of those with a SUD lose)

Experiment 2: count the children and every step gains

	Screen alone	+ π_V clears	+ price clears (main)
Newborn's CEV, % of lifetime consumption	2.82	6.45	**2.57**
Parent's own gain, %	−1.19	2.42	0.69
Drug use, change against Housing First, %	−43	−42	−21
Homelessness, change against Housing First, %	−8	−20	−11
π_V, % (benchmark 6.75)	6.75	9.24	7.80

- Screen alone: parents lose about what they lose without children, but the child gains more than its parent loses
- Recycling the savings into vouchers makes parents gain too
- The cheaper drug (price −5 %) draws some users back, which takes part of the gain away

Main experiment: every outcome falls, childhood homelessness included



- Drug use 8.92 → 7.07 %; homelessness 0.62 → 0.55 %; children in a shelter 1.03 → 0.92 %
- π_V +16 %, to 7.80 %; drug price −5 %; treatment among those with a SUD 24.6 → 29.0 %

Why homelessness falls

1. Households that stop using spend less on drugs, so more of them can house themselves
2. Less SUD means less unemployment, and so less homelessness
3. The vouchers revoked from households with a SUD go, through a higher π_V, to other low-income households

Among households aged 26 to 50, the years a child lives at home, homelessness also falls by 11 %

$\Rightarrow$ no trade-off between the externalities: by cutting use, the screen keeps more children out of a shelter than it sends to one

Where the gain comes from

$$\Delta W = \underbrace{(1-\omega_c)\Delta C}_{\text{childhood externality}} + \underbrace{(1-\omega_c)\Delta E}_{\text{earnings externality}} + \underbrace{\beta^{J_K}\Delta P}_{\text{own adult life}} + \underbrace{\omega_c(\Delta C + \Delta E)}_{\text{counted by the parent}}$$

	Both held	π_V clears	Both clear
Newborn's gain ΔW, % of lifetime consumption	2.82	6.45	**2.57**
Childhood its parent does not count	2.20	3.47	1.50
Productivity at entry its parent does not count	1.06	1.00	0.46
Own adult life under the policy	−0.75	1.54	0.43
What its parent does count	0.31	0.44	0.19
Externality, % of the gain	116	69	**76**
Parent's own gain, % of its lifetime consumption	−1.19	2.42	0.69

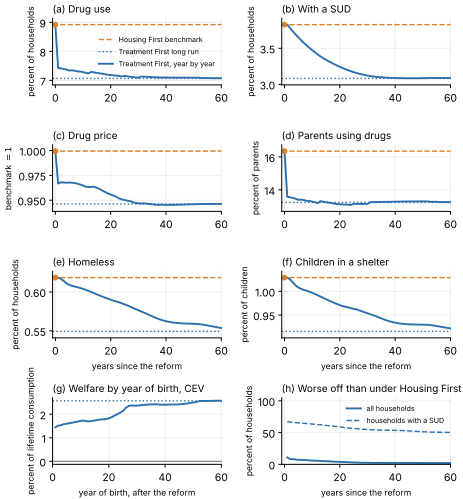
Table 6 of the paper. The childhood part is the largest: it accrues every year at home; the penalty reaches the child once.

Who gains, and why trials point the other way

- 99.9 % of households entering adult life are better off: the screen costs them nothing unless they choose to use
- 74 % of all households are better off and 3 % worse off; 24 % of those with a SUD are better off
- What a trial sees: those with a SUD when the reform lands lose; 74 % are worse off with the price and π_V held, as in a trial too small to move either, 67 % once both clear
- What it cannot see: the households the condition deters from ever starting, the vouchers its savings buy for others, and the children's adult lives

$\Rightarrow$ the model agrees with the micro evidence on the households it follows; the macro effects overturn the verdict

The transition from Housing First to Treatment First



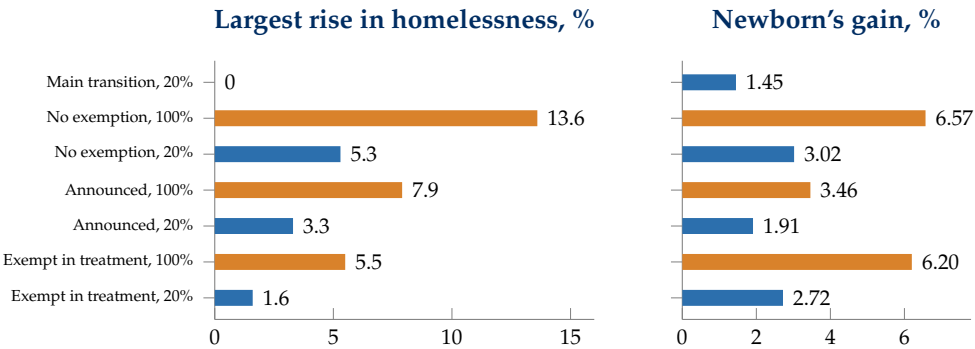
- Unexpected, permanent reform in year 1; all cohorts alive; p_v and π_V clear every year
- 81 % of the fall in drug use happens in year 1; SUD falls slowly (half by year 11)
- Homelessness 0.62 $\rightarrow$ 0.60 (year 10) $\rightarrow$ 0.55 %; children in a shelter 1.03 $\rightarrow$ 1.00 $\rightarrow$ 0.92 %: neither ever rises
- Children born at the reform gain 1.45 %; 11 % of adults alive at the reform lose

Why homelessness does not rise in the first years

1. **Few lose a voucher**: a household with a SUD keeps it by entering treatment and not using, and the screen catches a user 20 % of the time; at most 0.13 % of households are excluded in any year
2. **Drug use falls at once**: households spend less on drugs and have more left for housing, and fewer acquire a SUD and the unemployment it brings
3. **Vouchers are recycled**: after the first three years, when it dips to 6.55 %, π_V stays above its Housing First level of 6.75 %

The first reason depends on design: it needs treatment and sobriety to be a real option for those who already have a SUD

Alternative transitions: those with a SUD cannot stop using



- Orange: the screen catches every user; blue: one in five. A five-period exemption in treatment holds the rise to 1.6 %, and newborns still gain more than in the main transition

Robustness

Change	Result
Detection at half the rate, 10%	Newborn +1.65%; use −14%, homelessness −6%
Those with a SUD cannot respond at all	Use −29%, homelessness −11%; newborn gains at least as much
Parent's weight on the child at 0.69 (Nishiyama, 2000)	Newborn +3.18%, 23% of it an externality
No earnings penalty for the child	Treatment First still raises welfare; the earnings externality is 18% of the gain
A disamenity of homelessness (Loubière et al., 2020)	No long-run gain moves by more than 0.14 pp
Log utility in the stylized model	All qualitative results hold

Conclusion

Should housing assistance require sobriety?

- Yes, if the externalities on children are counted; no, if the case rests on moral hazard

Three lessons

1. Both sides of the debate leave out what matters most: the children of the households a sobriety requirement deters, in what they consume, whether they become homeless and what they earn as adults
2. The model agrees with the trials on the households they follow, who lose from the condition; the macro effects the trials cannot capture overturn the conclusion
3. Homelessness does not fall along the way unless treatment and sobriety are a real option for those who already have a SUD; a temporary exemption in treatment keeps childhood homelessness from ever rising

Thank you

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Backup: long-run results in full (Table 5)

	No externalities					With externalities				
		Treatment First					Treatment First			
	Housing First	Both held	π_V clears	Price clears	Both clear	Housing First	Both held	π_V clears	Price clears	Both clear
Adults										
Drug price, benchmark = 1	1.03	1.03	1.03	0.99	0.99	1.00	1.00	1.00	0.94	0.95
Voucher probability π_V	6.75	6.75	6.81	6.75	6.85	6.75	6.75	9.24	6.75	7.80
Drug use	8.97	6.48	6.48	7.47	7.46	8.92	5.08	5.19	7.10	7.07
Carrying a SUD	3.85	2.82	2.82	3.25	3.25	3.83	2.22	2.28	3.09	3.08
Homelessness	0.60	0.57	0.57	0.59	0.59	0.62	0.57	0.49	0.59	0.55
SUD share of the homeless	20.6	15.1	15.1	16.8	16.8	20.2	12.6	11.9	15.9	15.6
In treatment, of those with a SUD	24.7	29.0	29.0	29.0	29.0	24.6	29.3	28.7	29.0	29.0
Children										
Living with a parent who uses drugs	16.44	12.15	12.14	13.97	13.96	16.35	9.53	9.76	13.29	13.24
Living with a parent who has a SUD	5.85	4.37	4.37	5.01	5.00	5.82	3.43	3.56	4.76	4.75
In a shelter	1.01	0.96	0.95	0.98	0.98	1.03	0.95	0.83	0.98	0.92
Welfare										
CEV of a newborn, percent of lifetime consumption	–	−1.17	−1.09	−0.89	−0.74	–	2.82	6.45	0.98	2.57
Share of the newborn's gain that comes in childhood	–	–	–	–	–	–	86	59	102	64
Households entering adult life at 26 better off	–	0	1	0	18	–	0	100	5	100
All households better off	–	0	58	1	63	–	0	74	1	74
Households with a SUD better off	–	0	0	13	13	–	0	17	14	24

Percent unless stated. Columns 2–5: without externalities, against column 1; columns 7–10: with externalities, against the benchmark in column 6. Detection rate 20%.

Backup: alternative transitions in full (Table 7)

		Homelessness		Children in a shelter		Newborn's
	p_{det}	Largest rise	Periods above	Largest rise	Periods above	gain
Main transition	20	none	none	none	none	1.45
No exemption	100	13.6 (2)	1–15	10.8 (2)	2–11	6.57
	20	5.3 (6)	2–17	3.1 (5)	2–11	3.02
Announced five periods ahead	100	7.9 (10)	7–17	4.4 (9)	7–13	3.46
	20	3.3 (12)	7–19	0.9 (10)	8–13	1.91
Treatment exempt, first five periods	100	5.5 (7)	7–14	2.0 (7)	7–10	6.20
	20	1.6 (11)	8–16	none	none	2.72

Largest rise above Housing First, percent, with the period it occurs in brackets. The drug price and π_V clear every period.